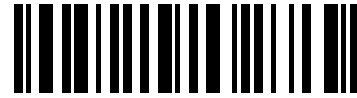


Name : **VIGNESH**
 PID : **2100228724**
 Age / Sex : 32 Y / Male
 Referrer : Self
 Branch : MADURAI - HUB



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Partial Test Report

INVESTIGATION / METHOD	RESULT	UNITS	BIOLOGICAL REFERENCE INTERVAL
HAEMATOLOGY			
NUTRICARE PACKAGE-1			
COMPLETE BLOOD COUNT(CBC) :EDTA WHOLE BLOOD			
RBC (Red Blood Cell Count) (Method : Electrical Impedance)	5.30	Million/cmm	4.5-5.5
Haemoglobin (HB) (Method : Spectrophotometry/Colorimetry-SLS)	15.6	gm/dL	13.0-17.0
PCV -(Haematocrit-Packed Cell Volume) (Method : Calculated)	45.6	%	40-50
MCV (Mean Corpuscular Volume) (Method : Derived from RBC Histogram)	86.2	fl	83-101
MCH (Mean Corpuscular Hemoglobin) (Method : Calculated)	29.4	pg	27-31
MCHC (Mean Corpuscular Hemoglobin Concentration) (Method : Calculated)	34.1	g/dl	31.5-34.5
Total WBC Count (Method : Fluorescent Flow Cytometry)	5460	cells/cumm	4000-10000
DIFFERENTIAL COUNT(DC) (Fluorescent Flow Cytometry-SF Cube)			
Neutrophils	53	%	40-80
Lymphocytes	37	%	20-40
Monocytes	6	%	2-8
Eosinophils	3	%	1-6
Basophils	1	%	0-1
Platelet Count (Method : Electrical Impedance)	2.41	Lakhs/cumm	1.5-4.5



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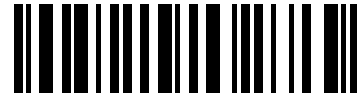


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Partial Test Report

INVESTIGATION / METHOD	RESULT	UNITS	BIOLOGICAL REFERENCE INTERVAL
Erythrocyte Sedimentation Rate(ESR) (Westergran Method)			
1 Hour (Specimen: EDTA WHOLE BLOOD)	08	mm/hr	<10
Absolute Basophils Count (Method : Fluorescent Flow Cytometry-SF Cube)	20	cells/cumm	20-100
Absolute Eosinophil Count (Method : Fluorescent Flow Cytometry-SF Cube)	180	cells/cumm	20-500
Absolute Lymphocyte Count (Method : Fluorescent Flow Cytometry-SF Cube)	2030	cells/cumm	1000-3000
Absolute Monocytes Count (Method : Fluorescent Flow Cytometry-SF Cube)	320	cells/cumm	200-1000
Absolute Neutrophil Count (Method : Fluorescent Flow Cytometry-SF Cube)	2910	cells/cumm	2000-7000
NRBC(Nucleated Red Blood Cells) (Method : Fluorescent Flow Cytometry-SF Cube)	0	%	<0
ANRBC(Absolute Nucleated Red Blood Cells) (Method : Fluorescent Flow Cytometry-SF Cube)	0	cells/cumm	<0
RDW-CV(Red Cell Distribution Width-CV) (Method : Derived from RBC Histogram)	12.40	%	11.6-14.0
RDW-SD(Red Cell Distribution Width-SD) (Method : Derived from RBC Histogram)	41.10	fl	39-46
MPV (Mean Platelet Volume) (Method : Derived from Platelet Histogram)	9.10	fl	6.5-12.0
PDW(Platelet Distribution Width) (Method : Derived from Platelet Histogram)	16.20	fl	9-14
Notes : *Unit and reference range changed as per hematology analyzer reporting			
PCT(Plateletcrit) (Method : Calculated)	0.23	%	0.19-0.39
PLCR(Platelet to Large Cell Ratio)	21.50	%	19.7-42.4

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Partial Test Report

INVESTIGATION / METHOD	RESULT	UNITS	BIOLOGICAL REFERENCE INTERVAL
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BIOCHEMISTRY.

NUTRICARE PACKAGE-1

Glycosylated HbA1c.
(HPLC)

HbA1c (Specimen: EDTA WHOLE BLOOD)	4.7	%	Non-Diabetic Level: < 5.7% Pre Diabetic : 5.7-6.4% Diabetic Level : >=6.5% Goal : 7.0%
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Notes : ADA (American Diabetes Association) guidelines on diagnosis of Diabetes Mellitus

Mean Blood Glucose Level (Specimen: EDTA WHOLE BLOOD)	88.2	mg/dL
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The A1C test results reflects your average blood sugar level for the past two to three months. It is a better reflection of how well your diabetes treatment plan is working overall. A committee of experts from the American Diabetes Association recommend that the A1C test be the primary test used to diagnose prediabetes, type 1 diabetes and type 2 diabetes.

GLUCOSE FASTING (FBS) (Method : Glucose Oxidase - Peroxidase) (Specimen: FLUORIDE PLASMA)	89	mg/d L	Normal: 74-100 Pre Diabetes: 100-125 Diabetes : >=126 ADA guidelines
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BIOCHEMISTRY

NUTRICARE PACKAGE-1

Blood Urea Nitrogen(BUN) (Method : Urease/GLDH) (Specimen: SERUM)	6.9	mg/dL	6.1-20.1
CREATININE (Method : Creatinine amidohydrolase) (Specimen: SERUM)	0.90	mg/dL	0.5-1.3
BUN/Creatinine Ratio (Specimen: SERUM)	7.6	Ratio	9-23
URIC ACID (Method : Uricase) (Specimen: SERUM)	5.3	mg/dL	3.6-7.5

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Partial Test Report

INVESTIGATION / METHOD	RESULT	UNITS	BIOLOGICAL REFERENCE INTERVAL
e-GLOMERULAR FILTRATION RATE (GFR) (Method : Calculated) (Specimen: SERUM)	116	ml/min/1.73 m2	> = 90 :Normal 60-89 :Mild Decrease 45-59 :Mild to moderate Decrease 30-44 :Moderate to severe Decrease 15-29 :Severe Decrease

Note:Estimates glomerular filtration rate by MDRD estimation

CALCIUM (Method : Arsenazo III) (Specimen: SERUM)	8.8	mg/dL	8.4-10.4
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LIPID PROFILE

CHOLESTEROL (Method : Cholesterol Oxidase,esterase,Peroxidase) (Specimen: SERUM)	144.0	mg/d L	Desirable :<200 Borderline high :200-239 High :>240
HDL CHOLESTEROL (Method : Direct) (Specimen: SERUM)	32.0	mg/d L	40-60
LDL CHOLESTEROL (Method : Enzymatic Clearance) (Specimen: SERUM)	98.6	mg/d L	Optimal :<100 Near Optimal/above Optimal:100-129 Borderline high :130-159 High :159-189 VeryHigh :>190 NCEP ATP III guidelines
TRIGLYCERIDES (Method : Lipase/Glycerol Dehydrogenase) (Specimen: SERUM)	67.0	mg/d L	Normal :<150 mg/dl Borderline high:150-199 mg/dl High :200-499 mg/dl very high :>500 mg/dl NCEP ATP III guidelines

Note: When serum Triglycerides are > 400 mg/dL, VLDL cholesterol calculation is not applicable, as the Friedewald formula is not valid beyond this range. Hence, VLDL value is not reported.

VLDL CHOLESTEROL (Method : Calculation) (Specimen: SERUM)	13.4	mg/d L	<30
Non-HDL Cholesterol (Method : Calculation) (Specimen: SERUM)	112.0	mg/dL	<130
CHO / HDL RATIO (Method : Calculation) (Specimen: SERUM)	4.5	Ratio	Optimal<3.5 Goal <5.0



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Partial Test Report

INVESTIGATION / METHOD	RESULT	UNITS	BIOLOGICAL REFERENCE INTERVAL
LDL/HDL RATIO (Specimen: SERUM)	3.1	Ratio	1.5-3.5 NCEP ATP III guidelines
TGL/HDL Ratio (Method : Calculated) (Specimen: SERUM)	2.1	Ratio	Ideal : <2.0 High risk : >4.0 Very high risk: 6.0

A lipid profile provides assessment of atherogenic risk and guides cardiovascular risk management. Borderline and high values should be correlated with patient's clinical history, risk factors (diabetes, hypertension, smoking, family history), and other investigations. Management includes lifestyle modification, dietary regulation, physical activity, and pharmacotherapy (e.g., statins, fibrates, niacin, PCSK9 inhibitors) where indicated.

SMALL DENSE LDL CHOLESTEROL(sdLDL) (Specimen: SERUM)	28	mg/dL	5.1 - 64.89
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LIVER FUNCTION TEST(LFT)

BILIRUBIN TOTAL (Method : Diazo) (Specimen: SERUM)	0.50	mg/d L	<1.2
BILIRUBIN DIRECT (Method : Diazo) (Specimen: SERUM)	0.10	mg/d L	0-0.3
BILIRUBIN INDIRECT (Method : Diazo) (Specimen: SERUM)	0.40	mg/d L	0.1 - 0.9
Aspartate aminotransferase(AST/SGOT) (Method : UV without P5P, IFCC Traceable) (Specimen: SERUM)	21.0	U/L	5-35
Alanine aminotransferase(ALT/SGPT) (Method : UV without P5P, IFCC Traceable) (Specimen: SERUM)	26.0	U/L	5-40
ALKALINE PHOSPHATASE (Method : AMP) (Specimen: SERUM)	68.0	U/L	53-128
GAMMA GT (Method : Glutamyltransferase) (Specimen: SERUM)	32.0	U/L	5-50
TOTAL PROTEIN (Method : Biuret) (Specimen: SERUM)	6.90	gms/dl	6.0-8.0
ALBUMIN (Method : Bromocresol Green(BCG)) (Specimen: SERUM)	4.30	gm/dL	3.5-5.2
GLOBULIN (Method : Calculation) (Specimen: SERUM)	2.8	g/dl	2.8-3.5



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Partial Test Report

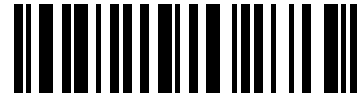
INVESTIGATION / METHOD	RESULT	UNITS	BIOLOGICAL REFERENCE INTERVAL
A/G RATIO (Specimen: SERUM)	1.5	Ratio	1.1-2.5
AST/ALT (Method : Calculated) (Specimen: SERUM)	0.81	%	1-1.19
UREA (Method : Urease) (Specimen: SERUM)	14.8	mg/d L	17-51

SEROLOGY

NUTRICARE PACKAGE-1

Hepatitis B surface antigen-Qualitative (HBs Ag-Qualitative) **NEGATIVE**
(Method : Chromatography)
(Specimen: SERUM)

All Reactive results must be confirmed by Neutralizing confirmatory test or by HBV DNA detection assay



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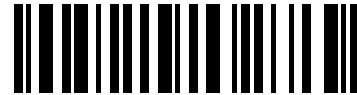
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Partial Test Report

INVESTIGATION / METHOD	RESULT	UNITS BIOLOGICAL REFERENCE INTERVAL
CLINICAL PATHOLOGY		
NUTRICARE PACKAGE-1		
URINE COMPLETE ANALYSIS (CUE)-URINE		
PHYSICAL EXAMINATION		
COLOUR (Method :Visual Inspection)	PALE YELLOW	Pale Yellow
CLARITY (Method :Visual Inspection)	CLEAR	Clear
CHEMICAL EXAMINATION		
PH (Method :MethylRed sodiumsalt, bromothymol blue)	5.0	4.6 - 8.0 [4.6-6.9 Acidic 7.0 Neutral 7.1-9.0 Alkaline]
SPECIFIC GRAVITY (Method :Bromothymol Blue indicator)	1.020	1.005-1.030
GLUCOSE-URINE(F) (Method :Glucose oxidase /Peroxidase)	NIL	Nil
PROTEIN (Method :Tetrabromophenol blue/ Sulphosalicylic Acid)	NIL	Negative
NITRITE (Method :p-arsanilicacid/diazonium)	NEGATIVE	Negative
KETONE (Method :Sodium nitroprusside reaction)	NEGATIVE	Negative
BILE PIGMENT&BILE SALT (Method :Diazonium)	ABSENT	Absent
UROBILLINOGEN (Method :2,4-dichloroaniline diazium salt)	NORMAL	Normal
BLOOD (Method :Pseudoperoxidase activity)	NEGATIVE	Negative
MICROSCOPIC EXAMINATION		
PUS CELLS (Method :Digital Microscopy)	2 - 5	0-5/hpf



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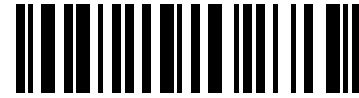
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Partial Test Report

INVESTIGATION / METHOD	RESULT	UNITS	BIOLOGICAL REFERENCE INTERVAL
EPITHELIAL CELLS (Method :Digital Microscopy)	1 - 3		0-5/hpf
RBC'S (Method :Digital Microscopy)	NIL		0-2/hpf
CRYSTALS (Method :Digital Microscopy)	ABSENT		Nil
HYALINE CASTS (Method :Digital Microscopy)	NIL		0-2 hyaline cast /lpf
PATHOLOGICAL CAST (Method :Digital Microscopy)	NIL		Nil
BACTERIA (Method :Digital Microscopy)	NIL		None
YEAST (Method :Digital Microscopy)	NIL		None
MUCUS (Method :Digital Microscopy)	ABSENT		Absent



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Partial Test Report

INVESTIGATION / METHOD	RESULT	UNITS	BIOLOGICAL REFERENCE INTERVAL
IMMUNOLOGY			
NUTRICARE PACKAGE-1			
INSULIN (F) (Method : CLIA) (Specimen: SERUM)	9.00	mIU/L	2.6-24.9
Notes : Mayo Clinical Laboratories published reference intervals.			
INSULIN RESISTANCE(HOMA IR) (Specimen: Fluoride(S-F))	1.1		0.7-2.0
BETA CELL FUNCTION. (Specimen: Fluoride(S-F))	108.3	%	
THYROID PROFILE TEST - TOTAL			
Total-T3 (Triiodothyronine-T3) (Method : CLIA) (Specimen: SERUM)	121.6	ng/dl	70-204
Total-T4 (Thyroxine T4) (Method : CLIA) (Specimen: SERUM)	9.9	ug/dl	4.5-11.7
TSH (Thyroid-stimulating hormone)-Ultra (Method : CLIA) (Specimen: SERUM)	1.800	uIU/ml	0.4-4.5

*Time of the day, stress, intense physical activity, certain medications, sleep deprivation, fasting and illness cause fluctuations in TSH levels.

*Hence it is advised to take the TSH test around the same time of the day and in the same manner (fasting/nonfasting).

End of the Report



Venu Anand
Dr.Mrs.A.VENU ANAND MD.,
Reg.No:62538
Pathologist